



February 22, 2026

Dr. Thomas Keane
Assistant Secretary for Technology Policy
Office of the Secretary
Department of Health and Human Services
Hubert H. Humphrey Building
200 Independence Avenue, SW
Washington, DC 20201

SUBMITTED ELECTRONICALLY VIA: <https://www.regulations.gov>

RE: Request for Information (RFI): Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Assistant Secretary Keane:

We appreciate the opportunity to respond to this RFI and to share our experience-based feedback from building and using artificial intelligence (AI) tools as part of clinical care delivery at scale. Cadence is on the front lines of a new era for healthcare where personalized, technology-enabled care empowers patients to take control of their health and reduces costs for Medicare, and we believe our experience is valuable for the Administration to consider. We also appreciate the “OneHHS” alignment reflected in this RFI and urge the Department of Health and Human Services (HHS) to use this moment to provide clear, practical policy pathways that enable responsible scaling of applied AI in real-world clinical workflows.

Cadence delivers Remote Physiologic Monitoring (RPM), Advanced Primary Care Management (APCM), and Chronic Care Management to nearly 90,000 patients nationwide, a majority of whom live in rural and underserved communities. AI is already central to this work: we use AI-enabled tools to scale high-touch coaching, streamline administrative tasks across electronic medical records (EMRs), and proactively identify patients who may benefit from medication titration – always with human-in-the-loop clinical oversight. In practice, the promise of “AI in clinical care” is to enable better clinical operations at scale – summarizing longitudinal trends, prioritizing patients who need attention, reducing alert fatigue, and embedding recommendations into existing EMR workflows so clinicians can act quickly and safely.

In response to the RFI’s focus on regulation, reimbursement, and research and development, we urge HHS to take a pragmatic, pro-innovation approach that:

- Removes outdated payment and program design barriers that discourage the appropriate use of automation and AI to deliver higher-value care.
- Aligns federal incentives so AI enhances productivity, reduces clinician burden, lowers costs, and improves outcomes – while protecting patients and maintaining public trust.
- Promotes interoperability and accountable measurement, including timely access to Medicare claims and standardized, API-based data exchange.

A. Key barriers to innovation and broader adoption of AI in clinical care

Key barriers include Medicare payment models that still equate value with documented minutes rather than clinical impact. Time-based requirements can unintentionally penalize tools that safely automate work and expand capacity, discouraging investment in AI-enabled care. Fee-for-service incentives can also discourage prevention and early intervention – precisely where AI-enabled monitoring and proactive outreach can produce the greatest downstream savings and improved outcomes.

Adoption is also constrained by the lack of a reimbursement pathway for the software and interoperability capabilities that make AI clinically usable. In practice, delivering clinically usable AI requires continuous engineering and operational investment, such as maintaining integrations that are not standardized, ensuring secure connectivity for tens of thousands of cellular devices, and sustaining 24/7 uptime across multiple EMRs and care settings.

Finally, fragmented data and delayed feedback loops slow validation and responsible scaling. Data remain siloed across EMRs, device platforms, and payers, and outcomes and utilization data – including claims– are not always available in a timely way. This limits the ability of providers and vendors to evaluate real-world performance, detect model drift, and iteratively improve tools based on outcomes and utilization.

B. Regulatory and programmatic actions HHS should prioritize

1. Support a predictable, risk-based posture that distinguishes between AI as a medical device and AI used for care delivery operations

HHS can accelerate adoption by promoting clear, consistent expectations across divisions and encouraging a proportionate, risk-based approach. For non-device clinical AI, HHS should publish practical guidance for governance, auditability, and change management (including when models are updated) rather than creating uncertainty that chills deployment of low-risk workflow tools. HHS should also provide clearer boundary guidance for clinical decision support and workflow tools so developers and health systems can understand when functionality is likely to trigger medical device oversight versus when it is better addressed through operational governance and quality management.

2. Enable responsible use of AI while protecting patient information

Trust will depend on strong privacy and security safeguards, clear patient-facing transparency, and practical guidance on procurement. HHS can support adoption by publishing model contractual provisions and procurement checklists for AI-enabled services that address data use, security controls, incident response, model monitoring, and shared accountability between vendors and providers.

C. Reimbursement and payment policy recommendations

1. Remove time-based barriers that discourage appropriate use of automation and AI in care management

As we articulated in Cadence’s prior response to the Health Technology Ecosystem Request for Information (CMS-0042-NC), eliminating or modernizing time-based requirements for team-based care management services such as RPM would encourage investment in technology that delivers more personalized and efficient care. Where automation safely reduces the staff time needed to perform tasks, payment should not penalize adoption of higher-value tools. CMS should consider payment approaches that recognize engagement and outcomes (not minutes) for technology-enabled care, paired with guardrails to prevent low-value utilization.

2. Expand access to modern care management models like APCM

APCM is an important step toward proactive longitudinal management that is not artificially constrained by minute-counting. HHS should remove restrictions that limit APCM to providers participating in specific value-based models, so more beneficiaries can access technology-enabled, and specifically AI-enabled, care – particularly in rural and underserved areas. Because APCM supports flexible team-based care delivery, it is well-suited to integrating AI that improves productivity and consistency (e.g., intake, summarization, and care gap identification), while preserving clinician accountability.

3. Modernize reimbursement to recognize the value of software, interoperability, and AI-enabled clinical support

HHS should create reimbursement mechanisms through both fee-for-service and value-based pathways that recognize the costs of software capabilities underpinning technology-enabled care, including interoperability, maintenance, and security. This approach can be paired with guardrails that raise the bar and protect Medicare from low-value utilization, such as requiring 24/7 clinical support and meaningful EMR integration so data and actions are visible in the patient’s chart.

4. Enable performance measurement through timely access to Medicare claims and outcomes data

HHS can promote accountability and public trust by expanding timely access to Medicare claims data for qualified evaluation and validation of outcomes for AI-enabled clinical care. Claims-based analyses can help quantify cost and utilization effects year-over-year and may be used to validate private sector analyses.

D. Interoperability and R&D priorities to accelerate responsible AI

Enhanced interoperability is foundational for clinical AI. HHS should continue to advance standardized, API-based exchange across settings and vendors, including by improving the consistency and completeness of longitudinal data available via APIs and supporting

benchmarking tools and representative datasets that enable evaluation across diverse populations. Interoperability policy should prioritize not just data access, but workflow-ready exchange that supports bi-directional communication (data in, actions out) and reduces custom integration timelines for providers. In particular, EMR providers should be mandated to provide read/write API access.

Cadence would welcome the opportunity to partner with HHS on demonstration projects and applied research that evaluate AI-enabled care management for chronic disease in Medicare populations, including studies that measure outcomes, clinician burden, and total cost of care when AI is embedded in real workflows.

Thank you for your consideration. We welcome the opportunity to engage further on the recommendations above and to share data supporting these comments. Should you have any questions or wish to meet, please contact Meryl Holt, General Counsel, at meryl@cadencerpm.com.

Sincerely,

A handwritten signature in grey ink, appearing to read "Altchek", is positioned above the printed name.

Christopher Altchek
Founder & CEO, Cadence